

A FRAMEWORK IN CHAPTERS

Chapter Twelve

Somatic Normativity

Ableism · Sanism · Lookism · Sizeism
A Fundamental Matrix

Somatic Normativity is the system that ranks able bodies above disabled bodies, sane minds above mad minds, conventionally-attractive faces above unconventional ones, and conventionally-sized bodies above unconventional ones. This chapter runs it through the five domains — from the prayer camp for the mentally ill to the beauty performance economy to the slow death of the disabled body in an inaccessible world.

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CHAPTER 12 OF AN ONGOING FRAMEWORK

Somatic Normativity

Somatic Normativity is the eleventh Fundamental Matrix. It has four sub-faces, handled here the same way Cisheteropatriarchy (Chapter 5) was handled as three. The four faces are **Ableism** (ranking able bodies above disabled bodies), **Sanism** (ranking sane minds above mad minds), **Lookism** (ranking conventionally-attractive faces above unconventional ones), and **Sizeism** (ranking conventionally-sized bodies above unconventional ones).

Somatic Normativity overlaps with Coloniality (Chapter 11) at the intersection of Western beauty standards (Colorism, Texturism) with Ableism and Lookism. But the two matrices are not the same. Coloniality ranks by colonial origin. Somatic Normativity ranks by bodily capacity and appearance as such — regardless of colonial origin. The framework separates them so that each logic can be studied and dismantled on its own terms.

A note on method. The Interpersonal, Disciplinary, and Structural sections take the naive approach — each triad described as if happening for the first time, with no networks, residues, or Canon referenced. The Hegemonic Domain section is where Canon and Practice finally enter the picture.

READING NOTE

Same convention as Chapters 1 through 11. Every paragraph is written to be tweetable. Where you see literal blanks (____), the framework is asking you to write yourself into it. The blanks are the difference between reading about Somatic Normativity and locating yourself inside it.

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SECTION 1

1. Definition: What Somatic Normativity Extracts, Enforces, and Removes

Somatic Normativity is the system that ranks able bodies above disabled bodies, sane minds above mad minds, conventionally-attractive faces above unconventional ones, and conventionally-sized bodies above unconventional ones. The term draws on Lennard Davis's *Enforcing Normalcy* (1995), which showed how the category of 'normal' was constructed in the 19th century alongside the statistical concept of the average, and how this construction created disability as deviation.

Rosemarie Garland-Thomson's *Extraordinary Bodies* (1997) showed how the disabled body has been constructed as spectacle in Western culture. Tobin Siebers's *Disability Theory* (2008) extended the analysis to show how disability is a complex social identity rather than a personal deficiency. Robert McRuer's *Crip Theory* (2006) connected disability studies to queer theory, showing how compulsory able-bodiedness parallels compulsory heterosexuality.

For Sanism, Peter Beresford's mad studies work and Jasbir Puar's *The Right to Maim* (2017) are central. Puar's concept of 'debility' — the deliberate production of bodily and mental damage in populations — is particularly relevant to Nigeria, where the chronic stress of multiple matrices produces mental illness that is then treated as personal failure. For Lookism, Deborah Rhode's *The Beauty Bias* (2010) documents the systematic advantage of the conventionally-attractive.

For Sizeism, Marilyn Wann's *Fat!So?* (1998) and Charlotte Cooper's *Fat Activism* (2016) supply the fat studies analysis. Susan Bordo's *Unbearable Weight* (1993) traces the cultural construction of body norms. Erving Goffman's *Stigma* (1963) supplies the foundational analysis of spoiled identity that runs across all four sub-faces. For the Nigerian context, Oye Gureje's work on Nigerian psychiatry and the WHO World Report on Disability (2011) document the structural conditions.

1.1 The Four Sub-Faces

Ableism is the system that ranks able bodies above disabled bodies. The wheelchair user who cannot enter the building because there is no ramp. The deaf person who cannot access the service because there is no interpreter. The blind person who cannot read the form because there is no Braille. The person with intellectual disability who cannot access education because the school is not equipped. Ableism treats the able body as the default and the disabled body as deviation.

Sanism is the system that ranks sane minds above mad minds. The person with depression who is told to 'pray about it.' The person with schizophrenia who is chained in the prayer camp. The person with anxiety who is told to 'be strong.' The suicide attempt survivor who is criminalised rather than treated. Sanism treats the sane mind as the default and the mad mind as deviation.

Lookism is the system that ranks conventionally-attractive faces above unconventional ones. The job applicant who is hired because they are good-looking. The news anchor who is promoted because they have the right face. The acid attack survivor who is hidden from public view. The person with facial scarring who is treated as deficient. Lookism treats the conventional face as the default and the unconventional face as deviation.

Sizeism is the system that ranks conventionally-sized bodies above unconventional ones. The fat person who is shamed in public. The thin person who is told to ‘eat more.’ The pregnant woman whose body is commented on by strangers. The postpartum woman who is told to ‘lose the baby weight.’ Sizeism treats the conventional size as the default and the unconventional size as deviation — though the conventional size itself varies across cultures and historical periods.

1.2 What Somatic Normativity Extracts

Somatic Normativity extracts five things. First, **disabled body labour for the beggar economy**: the visible disability that is used to solicit alms, with the disabled person positioned as the object of pity rather than the subject of rights. Second, **mad mind labour for the prayer camp economy**: the mental illness that is treated as spiritual possession, with the mad person channelled to the prayer camp where the prophet extracts income from the family in exchange for ‘deliverance.’

Third, **unconventional face labour for spectacle**: the unconventional face that is stared at, photographed without consent, or used as entertainment. Fourth, **unconventional size labour for the diet and gym economy**: the body that is told it must change, with the diet industry, the gym industry, and the cosmetic surgery industry extracting money from the subject’s desire to conform.

Fifth, **reproductive body labour for beauty performance**: the pregnant body, the postpartum body, the ageing body — each subjected to performance standards that extract labour, money, and bodily autonomy from the subject. Each of these is tangible and consequential.

1.3 What Somatic Normativity Enforces

Somatic Normativity enforces five standards. First, the **able body standard**: the able body is the default, and the built environment is designed for it. Stairs instead of ramps. Spoken language instead of sign. Print instead of Braille. Standing instead of sitting. The disabled body is treated as a problem to be accommodated, not as a default user.

Second, the **sane mind standard**: the sane mind is the default, and the social environment is designed for it. Rational decision-making is required for legal standing. Emotional stability is required for professional life. Cognitive consistency is required for credibility. The mad mind is treated as unreliable, dangerous, or deficient.

Third, the **conventional face standard**: the conventional face is the default in media, in hiring, in social life. Fourth, the **conventional size standard**: the conventional size is the default, with fat and thin bodies treated as deviation. Fifth, the **productive body standard**: the body must be productive — able to work, to reproduce, to perform — and the body that cannot is treated as burden.

1.4 What Options Somatic Normativity Removes

Concretely, Somatic Normativity removes the option to enter public buildings without an able body. The wheelchair user who cannot enter the bank because there is no ramp loses the option to banking. The deaf person who cannot access the court because there is no interpreter loses the option to legal voice. The blind person who cannot read the prescription because there is no Braille loses the option to health.

It removes the option to be mentally ill without social cost. The person with depression who is told to ‘pray about it’ loses the option to medical treatment. The person with schizophrenia who is chained in the prayer camp loses the option to liberty. The suicide attempt survivor who is criminalised under the Criminal Code loses the option to compassion. Section 327 of the Criminal Code criminalises attempted suicide with up to one year’s imprisonment.

It removes the option to have an unconventional face without social cost. The acid attack survivor loses the option to public life. The person with facial scarring loses the option to media representation. It removes the option to have an unconventional size without social cost. The fat person loses the option to respectful healthcare. The thin person loses the option to be taken seriously. Each removal is enforced by the built environment, by the medical profession, by the media, and by the triads below.

SECTION 2 · NAIVE APPROACH

2. The Interpersonal Domain

The Interpersonal Domain contains five enforcement triads and five extraction triads. Each is described here as if happening for the first time — no networks, no residues, no Canon. Just the act, the conforming subject, the complicit witness, and (for extraction triads) the Non-Extracting Beneficiary.

3.1 Enforcement Triad: Able Body Standard

The enforcer is the person or institution that demands the able body as the default. The architect who designs the building without a ramp. The event organiser who does not provide an interpreter. The employer who requires standing for the job. The school that does not accommodate the disabled child. The enforcement is performed by the built environment, by the service design, by the job description, or by the school policy.

The subject conforms by being able — or by not accessing the building, the service, the job, the school. The conforming act is the able body itself — the subject climbs the stairs, hears the spoken word, reads the print, stands for the job. The disabled subject is the subject who cannot conform and therefore cannot access.

The options the subject loses are concrete. The wheelchair user who cannot enter the bank loses the option to banking, which is to say, loses the option to economic participation. The deaf person who cannot access the court loses the option to legal voice. The blind person who cannot read the form loses the option to government services. The person with intellectual disability who cannot access education loses the option to development.

The witness is the able-bodied person who watches the able body enforcement and says nothing, or who themselves climb the stairs and so model compliance. The Non-Extracting Beneficiary is the able-bodied consumer who benefits from the able-bodied design without personally commanding it.

3.2 Enforcement Triad: Sane Mind Standard

The enforcer is the person or institution that demands the sane mind as the default. The court that requires rational decision-making for legal standing. The employer that requires emotional stability for professional life. The family that requires cognitive consistency for credibility. The hospital that requires cooperation for treatment. The enforcement is performed by the legal standard, by the employment policy, by the family judgment, or by the medical protocol.

The subject conforms by being sane — or by hiding their madness, by performing sanity, by suppressing their symptoms. The conforming act is the sane performance itself — the subject makes rational decisions, performs emotional stability, maintains cognitive consistency, cooperates with

treatment. The mad subject is the subject who cannot conform and therefore loses legal standing, employment, credibility, or treatment.

The options the subject loses are concrete. The person with depression who is told to ‘pray about it’ loses the option to medical treatment, which is to say, loses the option to health. The person with schizophrenia who is chained in the prayer camp loses the option to liberty. The suicide attempt survivor who is criminalised loses the option to compassion. The person with anxiety who is told to ‘be strong’ loses the option to support.

The witness is the sane person who watches the sane mind enforcement and says nothing, or who themselves perform sanity and so model compliance. The Non-Extracting Beneficiary is the prayer camp prophet who profits from the mad mind extraction without personally commanding the sane mind standard.

3.3 Enforcement Triad: Conventional Face Standard

The enforcer is the person or institution that demands the conventional face as the default. The casting director who selects the conventionally-attractive actor. The employer who hires the conventionally-attractive applicant. The media house that promotes the conventionally-attractive presenter. The family that prefers the conventionally-attractive spouse. The enforcement is performed by the casting decision, by the hiring decision, by the promotion decision, or by the marriage approval.

The subject conforms by having the conventional face — or by modifying the unconventional face through cosmetics, through surgery, through hiding. The conforming act is the face itself — the subject presents the conventional face to the camera, to the interview, to the public. The unconventional-faced subject is the subject who cannot conform and therefore loses media representation, employment, or social standing.

The options the subject loses are concrete. The unconventionally-attractive applicant loses the option to employment, which is to say, loses the option to economic participation. The acid attack survivor loses the option to public life. The person with facial scarring loses the option to media representation. The loss compounds across the life.

The witness is the conventionally-attractive person who watches the conventional face enforcement and says nothing, or who themselves have the conventional face and so model compliance. The Non-Extracting Beneficiary is the cosmetic industry that profits from the unconventional face extraction without personally commanding the standard.

3.4 Enforcement Triad: Conventional Size Standard

The enforcer is the person or institution that demands the conventional size as the default. The media house that casts the thin actor. The employer that comments on the employee’s size. The family that pressures the member to gain or lose weight. The healthcare provider that dismisses the fat patient’s symptoms as ‘just your weight.’ The enforcement is performed by the casting decision, by the workplace comment, by the family pressure, or by the medical dismissal.

The subject conforms by having the conventional size — or by modifying the unconventional size through diet, through exercise, through surgery. The conforming act is the size itself — the subject presents the conventional size to the camera, to the workplace, to the public. The unconventional-sized subject is the subject who cannot conform and therefore loses media representation, employment, or medical care.

The options the subject loses are concrete. The fat person who is dismissed by the doctor loses the option to accurate diagnosis, which is to say, loses the option to health. The thin person who is told to ‘eat more’ loses the option to bodily autonomy. The postpartum woman who is told to ‘lose the baby weight’ loses the option to recovery. Each loss is enforced by the media, by the medical profession, by the family.

The witness is the conventionally-sized person who watches the conventional size enforcement and says nothing, or who themselves have the conventional size and so model compliance. The Non-Extracting Beneficiary is the diet and gym industry that profits from the unconventional size extraction without personally commanding the standard.

3.5 Enforcement Triad: Productive Body Standard

The enforcer is the person or institution that demands the body be productive. The employer who requires the worker to stand for eight hours. The family that requires the mother to produce children. The state that requires the citizen to be economically active. The school that requires the child to sit still and perform. The enforcement is performed by the job description, by the family expectation, by the welfare requirement, or by the school policy.

The subject conforms by being productive — the body stands, produces, reproduces, performs. The conforming act is the productivity itself. The body that cannot produce — the disabled body, the sick body, the ageing body, the mad body — is the body that cannot conform and therefore loses employment, family standing, welfare, or education.

The options the subject loses are concrete. The disabled worker who cannot stand loses the option to employment. The sick worker who cannot produce loses the option to income. The ageing worker who cannot perform loses the option to respect. The mad worker who cannot cooperate loses the option to treatment. The loss compounds with Productivism (Chapter 8), which enforces the standard that one must be productive at all.

The witness is the productive person who watches the productive body enforcement and says nothing, or who themselves are productive and so model compliance. The Non-Extracting Beneficiary is the employer who profits from the productive body extraction without personally commanding the standard.

3.6 Extraction Triad: Disabled Body for Beggar Economy

The extractor is the person or network that positions the disabled body to solicit alms. The family member who takes the disabled child to the intersection to beg. The syndicate that controls the beggar

economy. The religious institution that uses the disabled body as evidence of need. The extraction is performed by positioning, by controlling the disabled person's movement, or by exploiting the disability for income.

The subject conforms by begging — or by being positioned at the intersection. What is extracted is the disability itself — the visible difference that solicits alms. The extraction transfers income from the public to the extractor, with the disabled person receiving a fraction or nothing.

The options the subject loses are concrete. The disabled person who is positioned at the intersection loses the option to education, which is to say, loses the option to development. The disabled person who is controlled by the syndicate loses the option to bodily autonomy. The disabled person who is exploited by the religious institution loses the option to dignity.

The witness is the person who gives alms to the disabled beggar and does not question the extraction. The Non-Extracting Beneficiary is the public that gives alms to assuage guilt without addressing the structural conditions that produce the beggar economy.

3.7 Extraction Triad: Mad Mind for Prayer Camp Economy

The extractor is the prayer camp prophet who requires the mad mind for the deliverance economy. The prophet who chains the mentally ill person and charges the family for deliverance. The camp that holds the mentally ill person for months or years. The extraction is performed by the religious authority, by the chaining, by the family's payment, or by the exploitation of the mad mind.

The subject conforms by being delivered — or by being chained, flogged, starved, and prayed over. What is extracted is the madness itself — the condition that justifies the deliverance. The extraction transfers income from the family to the prophet, with the mad person receiving chains instead of treatment.

The options the subject loses are concrete. The mentally ill person who is chained in the prayer camp loses the option to medical treatment, which is to say, loses the option to health. The person who is held for months loses the option to liberty. The person who is flogged loses the option to bodily safety. The family that pays loses the option to economic security.

The witness is the person who knows of the prayer camp extraction and does not intervene, or who themselves send a family member to the camp. The Non-Extracting Beneficiary is the religious institution that receives the tithe from the family without personally commanding the extraction.

3.8 Extraction Triad: Unconventional Face for Spectacle

The extractor is the person or institution that uses the unconventional face as spectacle. The photographer who takes the picture without consent. The media house that broadcasts the unconventional face as entertainment. The social media user who shares the unconventional face as meme. The circus-style display of the extraordinary body that Garland-Thomson documented.

The subject conforms by being displayed — or by hiding. What is extracted is the face itself — the visible difference that is used as spectacle. The extraction transfers attention and income to the extractor, with the unconventional-faced person receiving nothing or humiliation.

The options the subject loses are concrete. The unconventional-faced person who is photographed without consent loses the option to privacy, which is to say, loses the option to dignity. The person who is shared as meme loses the option to self-representation. The person who is displayed as spectacle loses the option to ordinary life.

The witness is the person who looks at the spectacle and does not question the extraction. The Non-Extracting Beneficiary is the social media platform that profits from the engagement without personally commanding the spectacle.

3.9 Extraction Triad: Unconventional Size for Diet/Gym Economy

The extractor is the industry that requires the unconventional size to desire change. The diet company that sells the weight-loss product. The gym that sells the membership. The cosmetic surgeon who sells the procedure. The pharmaceutical company that sells the diet pill. The extraction is performed by the marketing, by the product design, by the social pressure.

The subject conforms by purchasing — the diet product, the gym membership, the procedure, the pill. What is extracted is the money and the bodily autonomy — the subject's resources and the subject's relationship to their own body. The extraction transfers income from the subject to the industry, with the subject receiving a product that often does not work.

The options the subject loses are concrete. The fat person who buys the diet pill loses the option to bodily health, because the pill damages the body. The thin person who is pressured to gain loses the option to self-acceptance. The postpartum woman who is pressured to lose weight loses the option to recovery. The loss compounds with the beauty performance labour extracted by Colorism and Texturism (Chapter 11).

The witness is the person who consumes the diet and gym industry's products and does not question the extraction. The Non-Extracting Beneficiary is the media that profits from the advertising without personally commanding the standard.

3.10 Extraction Triad: Reproductive Body for Beauty Performance

The extractor is the system that requires the reproductive body to perform beauty. The maternity industry that sells the 'bump-friendly' wardrobe. The postpartum industry that sells the 'recovery' programme. The anti-ageing industry that sells the 'youth-restoring' cream. The extraction is performed by the marketing, by the product design, by the social pressure on the reproductive body.

The subject conforms by performing — the pregnant body that performs the 'glowing' pregnancy, the postpartum body that performs the 'bouncing back,' the ageing body that performs the 'ageing gracefully.' What is extracted is the labour and cost of beauty performance on the reproductive body.

The options the subject loses are concrete. The pregnant woman who is expected to perform the ‘glowing’ pregnancy loses the option to rest, which is to say, loses the option to bodily recovery. The postpartum woman who is expected to ‘bounce back’ loses the option to bond with the child. The ageing woman who is expected to ‘age gracefully’ loses the option to age authentically.

The witness is the person who watches the reproductive body performance and does not question the extraction. The Non-Extracting Beneficiary is the maternity, postpartum, and anti-ageing industries that profit from the standard without personally commanding it.

Nascent forms of this system include: ‘Cripple’, ‘Stand up’, The lack of ramps, The lack of sign language interpreters, The pity gaze, The ‘overcoming’ narrative, The beggar economy that exploits disabled bodies, The hidden child, The prayer camp for the disabled child, The miracle cure industry, ‘Mad’, ‘You need your head examined’, ‘Have you taken your medication?’, The dismissal of the angry as ‘unstable’, The prayer camp for the mentally ill, The lack of mental health services, The stigma against therapy, The belief that mental illness is spiritual, The belief that mental illness is weakness, The dismissal of suicide attempt survivors, ‘Beautiful’, The acid attack survivor, The facial scarring stigma, Tribal marks as both identity and stigma, The plastic surgery industry, The skin-clearing industry, ‘Fat’, Fat-shaming, Thin-shaming, The preference for the thin in media, The diet industry, The gym industry, The body-shaming of the pregnant, The body-shaming of the postpartum, The body-shaming of the ageing.

SECTION 3 · NAIVE APPROACH

3. The Disciplinary Domain

When the ten triads above link into networks, Somatic Normativity scales. The link can be hierarchical (the enforcer at one level is the subject at the level above) or community-based (the witness at one triad becomes the enforcer at another). Both forms are visible in every Nigerian institution that deals with bodies and minds.

4.1 Disciplinary Hierarchy — Nested Stars

A Disciplinary Hierarchy is a chain of nested triads. The enforcer at one level is the subject at the level above. Each middle position is simultaneously subject above and enforcer below. Somatic Normativity spreads through this form in every Nigerian institution that organises bodily capacity and appearance.

Peak Triad-Node

At the peak is the triad where the highest able, sane, conventional authority enforces on the layer below. In the medical hierarchy, the peak is the Chief Medical Director enforcing on consultants, who enforce on registrars, who enforce on nurses, who enforce on the psychiatric patient. In the disability services hierarchy, the peak is the Federal Ministry of Humanitarian Affairs enforcing on the National Commission for Persons with Disabilities, which enforces on state disability offices, which enforce on the disabled person.

In the beauty and media hierarchy, the peak is the casting director enforcing on the model, who enforces on the audience. In the body size hierarchy, the peak is the gym instructor and fitness influencer enforcing on the dieter. In the religious healing hierarchy, the peak is the General Overseer enforcing on the prophet, who enforces on the prayer camp inmate. The peak extracts disabled body labour, mad mind labour, beauty performance labour, and body modification labour from the layer below.

Middle Triad-Nodes

The middle nodes are where the chain's character is clearest. The consultant psychiatrist who defers to the CMD in the morning demands deference from the registrar in the afternoon. The state disability officer who defers to the National Commission in the morning demands deference from the disabled person in the afternoon. The conventionally-attractive model who defers to the casting director demands the conventional face from the aspiring model.

The middle is where Somatic Normativity reproduces. The disabled subject of today is the enforcer of tomorrow — not as the medical professional, but as the family member who hides the disabled child, the church member who recommends the prayer camp, the friend who recommends the diet. The middle position teaches the subject to enforce the standard on those below. By the time the Nigerian

becomes a parent, they have been disciplined by Somatic Normativity for decades.

Bottom Triad-Node

At the bottom is the triad where the lowest-ranked enforcer enforces on the most vulnerable body or mind. The nurse enforcing on the psychiatric patient. The prayer camp prophet enforcing on the mad inmate. The family member enforcing on the disabled child. The casting assistant enforcing on the aspiring actor. The gym member enforcing on the new joiner. The bottom is where the system's violence is most concentrated.

4.2 Disciplinary Community — Distributed Network

A Disciplinary Community is a distributed network. There is no peak, no chain of command. There is only circulation. The witness of one Somatic Normativity triad becomes the enforcer, subject, or witness of another. Somatic Normativity spreads laterally through any person who has witnessed bodily or mental enforcement without dissenting.

The Family Pressure Networks

The family pressure networks circulate Somatic Normativity laterally across the Nigerian family. The aunt who asks 'when are you losing the baby weight?' The uncle who asks 'why is your child not walking yet?' The grandmother who recommends the prayer camp for the depressed cousin. The mother who recommends the bleaching cream (overlapping with Coloniality, Chapter 11) for the dark-skinned daughter. The networks reproduce the standards across the household.

The Prayer Camp Networks

The prayer camp networks circulate Sanism laterally across the Nigerian religious community. The testimony of the 'delivered' mad person. The recommendation of the prophet. The sharing of the camp's address. The networks reproduce the prayer camp economy across the Christian community, with similar networks in the Islamic healing tradition.

The Gym and Diet Culture Networks

The gym and diet culture networks circulate Sizeism laterally across the Nigerian professional class. The WhatsApp group that shares the diet plan. The Instagram influencer who posts the transformation. The colleague who comments on the weight loss. The networks reproduce the conventional size standard across the professional class.

The Social Media Beauty Influencer Networks

The social media beauty influencer networks circulate Lookism and Colorism (Chapter 11) laterally across the Nigerian youth. The Instagram model who sets the face standard. The TikTok influencer who sets the makeup standard. The YouTube vlogger who sets the skincare standard. The networks reproduce the conventional face standard across the digital public.

The Medical Professional Networks

The medical professional networks circulate Somatic Normativity laterally across the Nigerian healthcare system. The doctor who dismisses the fat patient's symptoms as 'just your weight.' The psychiatrist who dismisses the poor patient's symptoms as 'cultural.' The nurse who treats the disabled patient as object. The networks reproduce the standards across the medical profession.

The Traditional Healer Networks

The traditional healer networks circulate Sanism and Ableism laterally across the Nigerian traditional healing community. The Babalawo who treats mental illness as spiritual attack. The traditional bone-setter who treats the physical injury. The networks reproduce the standards across the traditional healing community, with both therapeutic and harmful effects.

SECTION 4 · NAIVE APPROACH

4. The Structural Domain

Run the Ship of Theseus test. When the original medical professionals and original prayer camp prophets are dead and replaced, when the networks are disbanded and reform, what is consistent? What outlives both individuals and groups? What forces new Nigerians to re-form into the same patterns — not because anyone instructs them to, but because the residue makes any other pattern costly or invisible?

Somatic Normativity leaves all three residues. Almost every Nigerian institution carries Bureaucracy, Stratification, and Classification from Somatic Normativity simultaneously.

5.1 Bureaucracy — Enduring Rules and Procedures

Somatic Normativity's bureaucracy is the codified bodily and mental standard. The Lunacy Act 1916, a colonial-era mental health law that governed Nigerian mental health for over a century, was only replaced by the Mental Health Act 2021. The Lunacy Act allowed for the institutionalisation of the 'lunatic' by a magistrate's order, with minimal due process. The Mental Health Act 2021 is a significant improvement but its implementation is still incomplete.

The Discrimination Against Persons with Disabilities (Prohibition) Act 2018 codifies the rights of disabled Nigerians, including accessibility, education, and employment. The Act established the National Commission for Persons with Disabilities. But the Act's implementation is still incomplete — most public buildings remain inaccessible, most schools remain unequipped, most employers remain non-compliant.

The lack of accessibility provisions in the National Building Code means that most buildings are constructed without ramps, without lifts, without accessible toilets. The lack of sign language in public services means that deaf Nigerians cannot access courts, hospitals, or government offices. The lack of mental health parity in insurance means that mental health treatment is not covered by most health insurance.

The criminalisation of attempted suicide under Section 327 of the Criminal Code means that the suicide attempt survivor is arrested rather than treated. The cosmetic industry regulation is minimal, allowing unsafe products to reach the market. The diet industry regulation is minimal, allowing unsafe products to reach the market. The prayer camp regulatory failure allows the chaining and flogging of the mentally ill to continue with impunity. The original colonial authors of the Lunacy Act are dead. The bureaucracy lives.

5.2 Stratification — Enduring Zero-Sum Rankings

Somatic Normativity's stratification is the daily experience of bodily and mental rank in every institutional encounter. **Able/disabled** — the able person ranks above the disabled in access, in employment, in social standing. **Sane/mad** — the sane person ranks above the mad in credibility, in legal standing, in social recognition.

Beautiful/ugly — the conventionally-attractive person ranks above the unconventionally-attractive in media, in hiring, in social life. **Thin/fat** — the conventionally-sized person ranks above the unconventionally-sized in media, in healthcare, in social standing. **Productive/unproductive** — the productive body ranks above the unproductive in employment, in welfare, in social respect.

Independent/dependent — the independent person ranks above the dependent in social respect, in policy design, in family standing. **Normal/abnormal** — the normal body and mind rank above the abnormal in every domain. The ranking is in the air; it does not need to be announced. The Nigerian hospital enforces stratification without anyone issuing an order: the doctor is sane, the patient is mad; the doctor is able, the patient is sick; the doctor is whole, the patient is broken.

5.3 Classification — Enduring Categorical Divisions

Somatic Normativity's classification is the set of categorical names that organise every Nigerian into bodily and mental categories. **Able** and **disabled** — the universal pair, with able as the default and disabled as the deviation. **Sane** and **mad** — the mental pair, with sane as the default and mad as the deviation.

Beautiful and **ugly** — the facial pair, with beautiful as the aspiration and ugly as the failure. **Thin** and **fat** — the size pair, with thin as the Western aspiration and fat as the Western failure, though this varies across Nigerian contexts. **Normal** and **abnormal** — the universal pair, with normal as the default and abnormal as the deviation.

Healthy and **sick** — the health pair, with healthy as the default and sick as the temporary deviation. **Whole** and **broken** — the bodily integrity pair, with whole as the default and broken as the loss. **Complete** and **incomplete** — the same pair in different language. The classification does the work before any individual enforcer shows up. A disabled person walking into a job interview is already classified; the interviewer does not need to demand the able body because the category 'disabled' already carries the exclusion. A mad person walking into a court is already classified; the judge does not need to demand sanity because the category 'mad' already carries the discreditation.

SECTION 5

5. The Hegemonic Domain: The Ascension Ritual

Is Somatic Normativity ascended? Yes on all three counts. It produces all three structural residues including Stratification (so Internalised Domination applies). It has working Canon and Practice mechanisms. The vast majority of Nigerians cannot imagine a society where disabled bodies are treated as default, where mad minds are treated as credible, where unconventional faces are treated as beautiful, where unconventional sizes are treated as normal — despite the fact that pre-colonial Nigerian societies had more complex relationships with all four.

6.1 Canon — Rewriting History

Canon is the present belief that the able body, sane mind, conventional face, and conventional size have always been the standards. Each sub-face has its own Canon. The Ableism Canon is that the able body is the default. The Sanism Canon is that the sane mind is the default. The Lookism Canon is that the conventional face is beautiful. The Sizeism Canon is that the conventional size is normal.

What is forgotten? Pre-colonial Nigerian societies had more complex relationships with disability. The blind diviner — the Babalawo training involves sight in some traditions, but there are documented cases of blind diviners in Yoruba tradition. The lame chief. The Albino as both stigmatised and sacred — in some Yoruba communities, the Albino was treated as a special being, in others as stigmatised. The relationship was complex, not the simple exclusion that the colonial Canon imposes.

Also forgotten: that pre-colonial Nigerian mental health was treated through community ritual rather than institutionalisation. The person with what we now call depression was treated through community ceremony, not chained in a cell. The person with what we now call psychosis was treated through divination and herbal medicine, not flogged in a prayer camp. The colonial state introduced the Lunacy Act 1916 and the institutionalisation model, displacing the community-based approach.

Also forgotten: that pre-colonial Nigerian beauty standards were more diverse. The Yoruba standard celebrated the ‘gbajumo’ — the well-formed person — with dark skin and full body. The Igbo standard celebrated the ‘nwanyi oma’ — the beautiful woman — with dark skin and healthy body. The Hausa standard celebrated the ‘mace mai kyau’ — the beautiful woman — with full body. The colonial encounter imposed the Western standard, which the Canon now presents as natural.

Also forgotten: that pre-colonial Nigerian body size standards were more diverse. The fuller figure was celebrated in many Nigerian cultures as a sign of health, wealth, and fertility. The thin body was treated as evidence of illness or poverty. The colonial encounter imposed the Western thin standard, which the Canon now presents as natural — though the fuller figure retains cultural authority in some contexts, creating a contradiction that many Nigerian women navigate daily.

Internalised Domination

Canon produces Internalised Domination because Somatic Normativity produces Stratification. The able person's entitlement to access is now natural, automatic, unquestionable. The able person who walks up the stairs does not experience this as privilege. They experience it as the natural order. The sane person's entitlement to credibility is natural. The conventionally-attractive person's entitlement to social standing is natural. The conventionally-sized person's entitlement to moral judgment is natural. Each entitlement grows with repetition.

Internalised Authority

Canon produces Internalised Authority in the somatic-normativity enforcer regardless of personal material gain. The right to police the disabled body. The right to dismiss the mad as unreliable. The right to comment on the unconventional face. The right to shame the unconventional size. Each is performed by ordinary Nigerians daily — the stranger who comments on the disabled person's body, the family member who dismisses the depressed cousin as 'weak,' the casting director who rejects the unconventional face, the aunt who shames the fat niece. The right to enforce feels natural, automatic, and legitimate.

Internalised Oppression

Canon produces Internalised Oppression in the disabled subject, the mad subject, the unconventional-faced subject, the unconventional-sized subject. The disabled person who believes they are a burden. The mad person who believes they are unreliable. The unconventional-faced person who believes they are deficient. The unconventional-sized person who believes they are morally failing. The teaching is silent; it is in the air.

The deepest form of this is the subject who has absorbed Somatic Normativity so thoroughly that they enforce it on themselves. The disabled person who says 'I don't want to be a burden' to explain why they don't ask for the accommodation they need. The mad person who says 'I should be stronger' to explain why they don't seek treatment. The unconventional-faced person who says 'I know I'm not beautiful' to explain why they don't apply for the visible role. The unconventional-sized person who says 'I know I need to lose weight' to explain why they accept the comment. The rebellion-capable subject has become the self-policing subject.

6.2 Practice — Rewriting Reality

Practice is the second mechanism of the Ascension Ritual. The interpersonal triad is repeated, again and again, until it becomes habit. The somatic-normativity enforcer forgets they can choose not to enforce. The disabled subject forgets they can rebel. The witness forgets they can dissent. Pierre Bourdieu called this *habitus* — the sediment of repeated practice in the body.

Rosemarie Garland-Thomson, in *Staring* (2009), showed how the stare at the extraordinary body becomes reflexive — the starrer does not decide to stare; the body stares. The disabled person who is stared at learns to expect the stare, to navigate the stare, to absorb the stare. The body learns. The mind stops questioning. By adulthood, the Nigerian has performed the able body, sane mind, conventional

face, and conventional size triads so many thousands of times that the alternative — accommodating the disabled, believing the mad, celebrating the unconventional face, accepting the unconventional size — feels like a kind of amputation. The body has been trained. Practice has rewritten reality.

Death — Fast and Slow

Practice produces Death. Fast death in Somatic Normativity is spectacular: the killing of disabled infants in some communities — the child with visible disability who is killed at birth or abandoned. The death of the mental patient in the prayer camp from chains, starvation, or exorcism violence — Human Rights Watch has documented cases of mentally ill Nigerians chained for years in religious and traditional healing centres, with deaths from treatable conditions.

The suicide of the person who cannot live with the unconventional face or size — the person who has internalised the beauty standard so deeply that their own body is unbearable. The acid attack — the violence that destroys the face as punishment. The honour killing of the unconventional-faced woman. The death from unsafe cosmetic procedure — the failed bleaching injection (overlapping with Coloniality, Chapter 11), the failed plastic surgery, the unlicensed buttock injection. The death from unsafe diet pill. The death from the gym injury — the steroid use, the overtraining, the untreated heart condition.

Slow death in Somatic Normativity is quiet. Chronic stress of living in an inaccessible world — the wheelchair user who has not slept properly in years because every trip requires planning, every building requires negotiation, every service requires advocacy. Mental health collapse from internalised madness — the person with depression who has internalised the ‘be strong’ standard so deeply that they cannot seek help until the body breaks down.

The slow death of the disabled body from lack of care — the disabled person who cannot access healthcare because the clinic has no ramp, because the doctor has no training, because the family has no resources. The slow death of the mad mind from lack of treatment — the person with schizophrenia who is chained in the prayer camp for years, their condition worsening with each month of untreated illness. The slow erasure of the unconventional face from media — the casting decisions, the editing decisions, the filtering decisions that remove the unconventional face from public view.

The slow erasure of the unconventional size from public life. The cumulative damage of beauty performance labour — the bleaching cream that damages the kidneys (overlapping with Coloniality), the relaxer that damages the scalp, the diet pill that damages the heart, the cosmetic surgery that goes wrong. The cumulative damage of body modification. The chronic stress of navigating a world that treats the body as deviation. The mental health collapse from internalised inferiority. None of these make the news. They are Somatic Normativity’s quiet tax.

Ruth Wilson Gilmore’s definition applies: Somatic Normativity produces group-differentiated vulnerability to premature death. The groups are the disabled, the mad, the unconventional-faced, the unconventional-sized, the unproductive body. The premature death is fast (infanticide, prayer camp death, suicide, acid attack, unsafe procedure, diet pill death, gym injury) or slow (inaccessible world stress, internalised madness, lack of care, lack of treatment, media erasure, beauty performance

damage, body modification damage, chronic stress, mental health collapse). Every ascended matrix kills. Somatic Normativity kills those whose body or mind does not fit the norm.

SECTION 6

6. When Did Somatic Normativity Rewrite History?

The framework's historical question: when did Somatic Normativity rewrite history in popular Nigerian living memory, such that the average Nigerian cannot recall a time when its structural residue was not present? This is not a history of how Somatic Normativity was built. It is a diagnosis of when the building was forgotten.

Pre-colonial Nigerian societies had more complex relationships with disability, madness, and body diversity. The blind diviner, the lame chief, the Albino as both stigmatised and sacred. The community-based treatment of mental illness through ritual and herbal medicine rather than institutionalisation. The diverse beauty standards that celebrated dark skin and full body. The diverse body size standards that celebrated the fuller figure as health, wealth, and fertility.

Colonial codification imposed the Western standards. The Lunacy Act 1916 introduced the institutionalisation model, displacing the community-based approach to mental health. The colonial school introduced the Western beauty standard through the curriculum and the missionary teaching. The colonial media introduced the Western face and body standards through advertising and cinema. The colonial medical profession introduced the Western model of disability as individual deficiency rather than social construction.

Post-independence replication continued the displacement. The Nigerian mental health system retained the colonial institutionalisation model. The Nigerian media retained the Western beauty standard. The Nigerian medical profession retained the Western model of disability. The Nigerian school retained the Western curriculum. The Mental Health Act 2021 and the Discrimination Against Persons with Disabilities Act 2018 are partial challenges to the colonial inheritance, but their implementation remains incomplete.

The tipping point in popular living memory: **circa 1916 for the Lunacy Act** (colonial-era mental health law); **circa 1960 for the consolidation of colonial-era beauty and body standards**. For Nigerians born after 1960 — the post-independence generation — Somatic Normativity has 'always been' in its current form. They cannot recall a time when the able body was not the default, when the sane mind was not the default, when the conventional face was not beautiful, when the conventional size was not normal.

The pre-somatic-normativity generation (born ~1890s-1910s) was the last to have living memory of pre-colonial relationships with disability, madness, and body diversity. By independence (1960), that generation had been displaced from policy-making by the colonial-trained elite, and the colonial somatic standards had become the natural order. The question 'is the able body natural?' had stopped being asked in popular memory by the 1960s. The question 'is the sane mind natural?' stopped being

asked by the 1960s. The question ‘is the conventional face natural?’ stopped being asked by the 1980s. The question ‘is the conventional size natural?’ stopped being asked by the 1990s, with the Western thin standard intensifying through the 2000s and 2010s.

SECTION 7

7. Fill in the Blank: A Self-Examination

The framework asks two questions of every subject of every ascended matrix. The blanks are for you. They are not rhetorical. The point of the framework is not to understand Somatic Normativity in the abstract. It is to locate yourself inside it, so that the next time the triad assembles around you — or around someone whose body or mind does not fit the norm — you can see it.

Question 1: When did Somatic Normativity rewrite your reality?

When did the system teach you that you could not rebel? The moment was: _____. The year was: _____. The person who first made you feel that challenging the bodily or mental standard was impossible was: _____. The place was: _____. What you wanted to say but did not was: _____. What you did instead was: _____. What you told yourself about why you did not say it was: _____. The bodily or mental standard you assumed in that moment was: _____. The form of address you used was: _____. What you lost, in that moment, that you have not got back since, was: _____. The sub-face of Somatic Normativity at the centre of the moment (Ableism, Sanism, Lookism, Sizeism) was: _____.

Question 2: How is Somatic Normativity causing your Death?

How is the system shortening your life right now? The form of Death I am most afraid of from Somatic Normativity is (fast/slow): _____. The specific way it is happening in my body is: _____. (For example: the chronic stress from years of navigating an inaccessible world; the insomnia from years of hiding my mental illness; the loss of self from years of being told my face is not beautiful; the loss of self from years of being told my body is not the right size; the breakdown I had in 20__ that I have never fully recovered from; the relationship I lost because I could not stop performing the bodily standard at home; the medical diagnosis I now live with because the cosmetic procedure went wrong; the medical diagnosis I now live with because the diet pill damaged my heart; the disabled family member I could not protect from the prayer camp; the mentally ill friend I could not help because the services were not there.) The matrix’s name for what is happening to me is: _____. My own name for it is: _____.

If you filled in the blanks honestly, you have just located yourself inside Somatic Normativity. The next time the triad assembles — the next time the building has no ramp, the next time the mad person

is chained, the next time the unconventional face is hidden, the next time the unconventional size is shamed — you will be able to see the system moving through you. That sight is the beginning of abolition. And if you are now the enforcer in the triad — the architect, the doctor, the casting director, the family member — the sight is also the beginning of your refusal to enforce.

SECTION 8

8. Endnotes & References

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Coming Next

Chapter 13 will take up Retributivism — the system that responds to harm with more harm. The framework's outline notes that nobody denies that rapists harm people, that paedophiles harm children, or that murder is a big deal, but that Retributivism is the oppressive system because it responds to harm with more harm rather than with restoration and healing. Retributivism overlaps with State Sovereignty (the prison as state apparatus) and with Rankism (the guard/prisoner hierarchy). The framework separates them so that the retributive logic specifically can be studied and dismantled.

If you are reading this chapter as part of the framework in progress, the order continues: Chapter 14 is Amatonormativity, Chapter 15 is Compulsory Existence / Suicidism, Chapter 16 is Meritism / The Significant, and so on through the Fundamental Matrices. Each chapter follows the same template established in Chapters 2 through 12. The naive approach in the first three domains (or its waiver, as in Chapter 11). The Ascension Ritual in the fourth. The historical memory trace. The fill-in-the-blank. The endnotes. The pattern is now set.